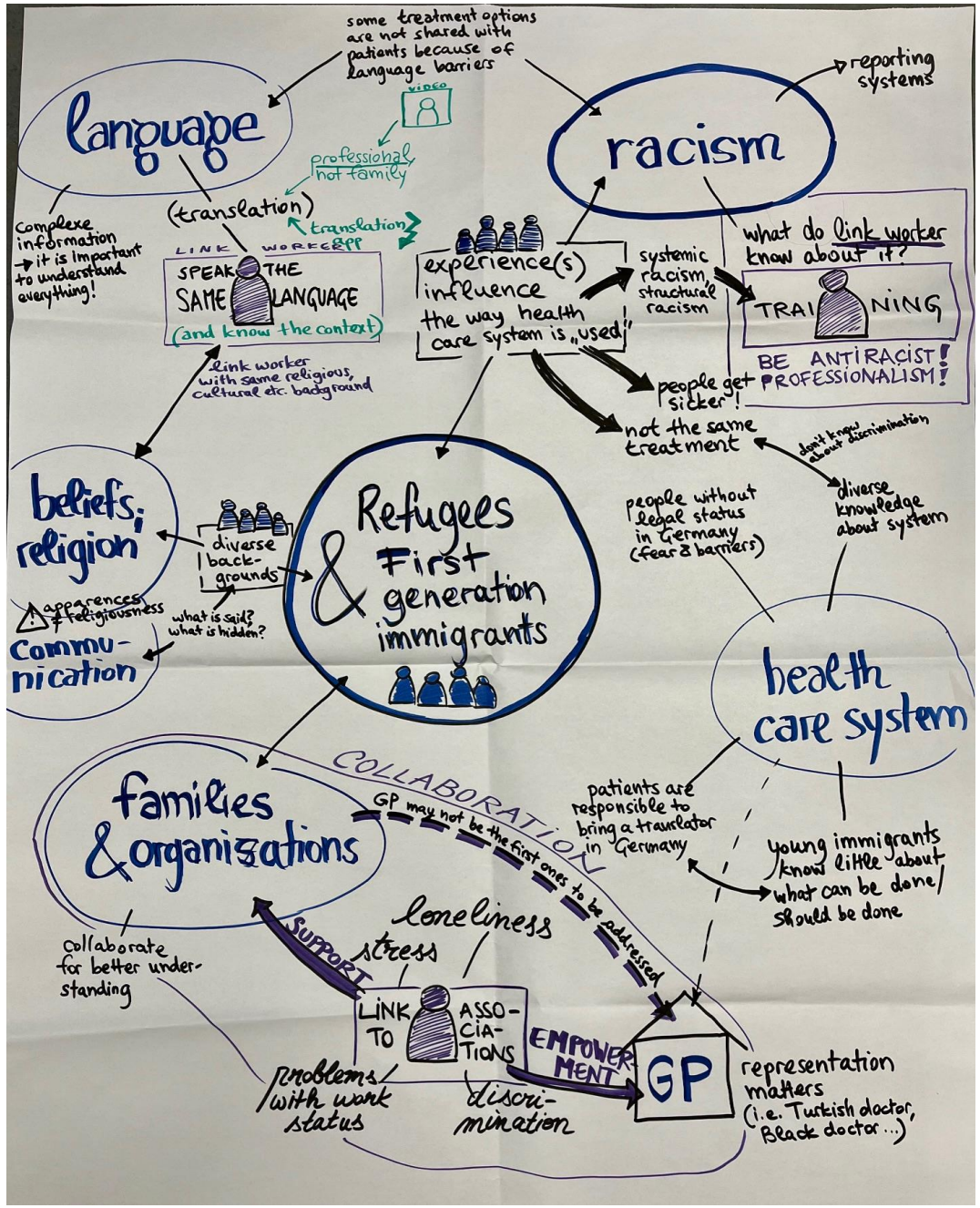
SP-EU Workshop 02/2024

Results and Ideas from Consultation of Representatives of Vulnerable Groups



February 19, 2024

Consultation with Representatives of refugees and first-generation immigrants - Results and Ideas

Representatives:

Eduardo Buanaissa (Educational Scientist, Landesnetzwerk afrikanischer Vereine e.V.)
Sevinç Kuzuoğlu (“Muslimisch gelesene Vielfalt im Gespräch”, Türkische Gemeinde in Deutschland)
Ngoc Anh Nguyen (Vereinigung der Vietnamesen in Berlin und Brandenburg e.V.)

February 20, 2024

Consultation with LGBTIQ - Results and Ideas

Representatives:
Tilly Tracy Reinhardt (StandUp – Fachberatung Antidiskriminierung, Schwulenberatung Berlin)
Gabriela Lünsmann (Lawyer, “Menschen und Rechte”, Association of Free Lawyers)
Eliana Sophie Wojtal (Medical Student, Queernetzwerk Charité)

Building trust

Challenges

- fear of discrimination when going to a doctor’s office
- Fear of explaining the own sexuality/identity over and over because there is no knowledge

Solutions/Ideas

- queermed as example for resource of LGBTIQ-friendly practices in Germany
- LW should be well-connected and have knowledge about the community

SP as support structure

Challenges

- SP should not lead to pathologization
- SP should be clearly communicated as support structure
- SP should not deter from medical treatment

LGBTIQ-specific that SP could answer

- connect to communities
  - i.e. social connectedness during transition, get to know other people who are at different stages of their transitioning process
  - isolation in rural communities
- empowerment in general

Other locales besides GP office for SP meetings?

- GP office as “neutral ground” makes sense
- home visits (although young LGBTIQ might not want their parents to know that they are queer so an SP visiting at home could be a problem)

Be mindful of different needs of LGBTIQ

Challenges

- transgender and non-binary people might need different sports groups than a cis-man
- intersectionality exists (i.e. being old and being queer)

Solutions/Ideas

- queermed as resource for LGBTIQ-friendly practices in Germany
- LW should be well-connected and have knowledge about the community

How can LW be trained to meet the group’s needs?

- include representation early in training
- have knowledge about (historical) background of queers in medical system
- get to know and work with community-related services: queer sports courses (i.e. *Seitenwechsel* in Berlin), NGO-run free health/consultation services (i.e. *Checkpoint BLN*)
- don’t be afraid of making mistakes! but how to apologize correctly when mistakes are made is important

February 20, 2024

Consultation with Older Adults

Representatives:
Francis Hoelzlsauer

Heterogenous Group

- very different physical and mental health states
- different means of participating in social and cultural life

Social Problems of Older Adults

- not “prepared” for retirement
- different partner status (married or not? widowed?)
- intersectionality exists also in old age (old and queer)
- loneliness
- substance abuse

Where should the LW meet the patient?

- participant would not want to be visited at home
- [group talked shortly about a project from Hamburg in which citizens aged 80+ receive a letter and are offered a “preventive home visit”]

What are your expectations from the LW?

- experiences from nurses and practice assistants should be taken into account*
- should tell older adults about existing offers, like sports programs, cultural programs

**: it is likely that the participant misunderstood the question – however, I liked the idea of nurses/PA´s/care-at-home-nurses perspective as they often have known the older patients for years and can make a good assessment of their physical/mental state and their needs (cf. how to identify older adults in precarious situations?); this could also include family members?*

Where should the LW meet the patient?

- participant would not want to be visited at home
- [group talked shortly about a project from Hamburg in which citizens aged 80+ receive a letter and are offered a “preventive home visit”]

Perception of the GP from an older adult’s perspective?

- COVID seems to have made older adults wary of doctors and the medical system

Ideas on how to address loneliness?

- visits from others should not be institutionalized/professionalized (i.e. an NGO coming to visit an older person at their home as a “job”)
- “being on my own” does not necessarily mean being lonely/feeling isolated
- take into account how society thinks of older adults [important for dissemination process of our results]

Are older people discriminated against?

- getting older, participant feels more sensitive / more irritated about things
- feelings of discrimination depend on own resilience

How to identify older adults in precarious situations?

- doctors should take time to make more “small talk”
- some problems are not talked about (i.e. financial problems)